

स्थौल्य (Sthaulya)

Introduction

Excessive accumulation of fat in the region of breast, hip, belly etc. of the body leading to various complications is identified as *Medovrddhi* or *Medo-Roga*. The patient of *Medo-Vrddhi* faces such indispositions as excessive sweating, hunger and thirst, he emits bad smell, feels weakness and difficulty in sexual intercourse and in movement. The indisposition may lead to many complications e. g. diabetes, heart disease, hemiplegia and arthritis.

स्थौल्य हेतु एवं संप्राप्ति (Sthaulya Hetu evam Samprāpti)

रसनिमित्तमेव स्थौल्यं काश्यं च । तत्र श्लेष्मलाहारसेविनोऽ-
ध्यशनशीलस्याव्यायामिनो दिवास्वप्नरतस्य चाम एवात्ररसो
मधुरतरश्च शरीरमनुक्रामन्नतिस्नेहान्मेदो जनयति । तदतिस्थौल्य-
मापादयति । तमतिस्थूलं क्षुद्रश्वासपिपासाक्षुत्त्वप्नस्वेदगात्रदौर्गन्ध्य-
क्रथनगात्रसादगद्वत्त्वानि क्षिप्रमेवाविशन्ति, सौकुमार्यान्मेदसः
सर्वक्रियास्वसमर्थः, कफमेदो निरुद्धमार्गत्वाच्चाल्पव्यवायो भवति
आवृतमार्गत्वादेव शेषा धातवो नाप्यायन्तेऽत्यर्थमतोऽल्पप्राणो
भवति, प्रमेहपिडकाज्वरभगन्दरविद्रधिवातविकारानामन्यतमं प्राप्य
पञ्चत्वमुपयाति । सर्व एव चास्य रोगा बलवन्तो भवन्त्या-
वृतमार्गत्वात् स्रोतसाम्, अतस्तस्योत्पत्तिहेतुं परिहरेत् ॥

S.S.Su. 15/32

मेदस्तु सर्वभूतानामुदरेष्वस्थिषु स्थितम् ।
अत एवोदरे वृद्धिः प्रायो मेदस्विनो भवेत् ॥
मेदसाऽऽवृतमार्गत्वाद्वायुः कोष्ठे विशेषतः ।
चरन् सन्धुक्षयत्यग्निमाहारं शोषयत्यपि ॥
तस्मात् स शीघ्रं जरयत्याहारमभिकाङ्क्षति ।
विकारांश्चाप्नुते घोरान् कांश्चित् कालव्यतिक्रमात् ॥
एतावुपद्रवकरौ विशेषादग्निमारुतौ ।
एतौ तु दहतः स्थूलं वनदावो वनं यथा ॥
मेदस्यतीव संवृद्धे सहसैवानिलादयः ।
विकारान् दारुणान् कृत्वा नाशयन्त्याशु जीवितम् ॥

M.Ni. 34/4-8

The etiological factors of the *Sthaulya* are as follows : Excessive eating, intake of heavy, cold, sweet, fatty food substances, *Kapha Vardhaka* foods & drinks and activities; lack of physical exercises, day sleep, lack of sexual intercourse and mental exercise, cheerfulness, intake of sweet substances by mother during pregnancy, defective genes & other *Dhātu* saturating measures.

स्थौल्य संप्राप्ति (Sthaulya Samprāpti)

Pathogenesis of *Sthaulya* described below :

Due to etiological factors *Kapha Doṣa* become aggravated & food remains uncooked & turns to more sweet, this *Rasadhātu* circulating throughout the body produces *Medas* because of its unctuousness is known as *Sthaulya*.

Sthaulya person suffers from mild dyspnoea, severe thirst, hunger, sleep, perspiration, bad smell from the body, snoring, debility of the body & stammering.

Due to softness of *Meda* the person becomes incapable of all activities; because of obstruction in the *Śukravaha Srotas* by *Kapha* & *Meda* leads to poor sexual pleasure; due to obstruction in other tissue channels the remaining *Dhātus* do not get sufficient nourishment, this leads to lesser strength in all activities. *Sthaulya* persons becomes victims of *Prameha*, *Prameha Piḍakā*, *Jvara*, *Bhagandara*, *Vidradhi*, *Vāta Vikara*, *Śvāsa*, *Udara roga* & *Urustambha* & such other diseases & death. All the above diseases become very powerful due to obstruction of the *Srotas*.

Abdomen & bones are the chief depositor of *Meda*, the belly gets enlarged in such individuals. The channels of *Vāta* become obstructed by the increased *Meda* inside the abdomen; *Vāta* then begins to act vigorously inside the *Mahāsrotas* as a result *agni* becomes powerful leading to voracious hunger and craves for more quantity of food. In due course of time more distressing conditions

prevails. Combination of both *Agni* & *Vāta* are more harmful just as forest fire destroys the forest, these destroy the body. Due to enhancement of *Meda* all the three *Doṣās* become aggravated & causes many serious complications including death.

Pathogenesis of Sthaulya due to increased Meda & Kapha According to Suśruta

Nidāna Sevana (Consumption of Etiological factors)

↓
Aggravation of *Kaphādi Doṣās*

↓
Disturbance of *Agni*

↓
Food remains *Āma* (uncooked) & turns to more sweet (*Madhura Avasthāpaka*)

↓
This *Adhya Rasadhātu* circulating throughout the body produces *Medas* because of its unctuousness

↓
Sthaulya.

Pathogenesis of Sthaulya due to increased Meda & Kapha Associated with Powerful Agni & Vāta

Nidāna Sevana (Consumption of Etiological factors)

↓
Aggravation of *Kaphādi Doṣās*

↓
Increase in quantity of *Meda* due to similar properties with *Kapha*

↓
Abdomen & bones are the chief depositor of *Meda* (the belly gets enlarged in such individuals)

↓
The channels of *Vāta* become obstructed by the increased *Meda* inside the abdomen

↓
Vāta then begins to act vigorously inside the *Mahāsrotas* as a result *agni* becomes powerful leading to voracious hunger and craves for more quantity of food

↓
Eat more quantity of food

↓
Sthaulya.

↓
In due course of time more distressing conditions prevail

↓
Combination of both *Agni* & *Vāta* are more harmful just as forest fire destroys the forest, these destroy the body

↓
Due to enhancement of *Meda* all the three *Doṣās* become aggravated & causes many serious complications and diseases

↓
Prameha, Prameha Pīḍakā, Jvara, Bhagandara, Vidradhi, Vāta Vikara, Śvāsa, Udara roga & Urustambha & such other diseases & death. (These diseases become very powerful due to obstruction of the *Srotas*)

स्थौल्य लक्षण (Sthaulya Lakṣaṇa)

मेदोमांसातिवृद्धत्वाच्चलस्फिगुदरस्तनः ।

अयथोपचयोत्साहो

नरोऽतिस्थूल

उच्यते ॥

M.Ni. 34/9

Person can be diagnosed as *Sthaulya* when his buttocks, abdomen & breasts begin to show movement due to excess accumulation of *Meda* in these areas. Contrary to his age there is a disproportionate in relation to build & enthusiasm (B.P.M. 39/9-10).

Kuṣṭha, Visarpa, Bhagandara, Jvara, Atisāra, Prameha, Arśa, Ślīpada, Apaci, Kāmalā may develop in *Sthaulya* persons. Due to the bad smell of sweat minute worms may develop in the skin (B.P.M. 39/9-10).

मेदस्यतीव संवृद्धे सहसैवानिलादयः ।

विकारान्दारुणान्कृत्वा नाशयन्त्याशु जीवितम् ॥

मेदोमांसातिवृद्धत्वाच्चलस्फिगुदरस्तनः ।

अयथोपचयोत्साहो

नरोऽतिस्थूल

उच्यते ॥

BPM. 39/8-9

When *Meda Dhātu* becomes greatly increased, then *Vātādi Doṣās* give rise to many dreadful diseases & destroy the life of the individuals.

Due to excess enhancement of *Meda* & *Māmsa* leading to pendulous movement of buttocks,

abdomen & breast, disproportionate increase of body parts & lack of enthusiasm to work is known as *Atisthūla*.

उपशय/पथ्य (Upaśaya/Pathya)

Persons indulging in physical exercise, worry, sexual intercourse, long walk, intake of honey, remaining awake at night and taking *Yava* and *Śyāmaka* as food gets rid of obesity. (B.R. 39/1)

If the patient wants to be free from obesity, then he should gradually increase *Asvapna* (remaining awake at night), sexual intercourse, physical exercise and mental activities. (B.R. 39/2)

The following measures and eatables are very much helpful in curing the problem of obesity: sorrow, labour, waking during night, enjoying sex, unguents, *Laṅgana* (fasting), facing the sun, riding elephant and horse, traveling, *Apatarpaṇa*, *Purātana*, *Vaiṇava*, *Koradūṣa*, *Kodarva*, *Śyāmaka*, *Nivāra*, *Priyaṅgu*, barley (*Yava*), *Kulattha*, gram (*Caṇaka*), lentil, *Mudga*, pigeon peas, honey, poppy of paddy, edibles and juices having bitter tastes, buttermilk, liquor, *Ciṅgata* variety of fishes, fried brinjal, *Triphalā*, *Guggulu*, *Lauha Bhasma*, *Trikaṭu*, mustard oil, cardamom, all dry materials, sesame oil, leafy vegetables, poultice of *Agara*, warm water, *Śilājatu*, drinking water before meals. (B.R. 39/67-71)

अनुपशय/अपथ्य (Anupaśaya/Apathya)

The following measures and diets are considered to be harmful for the patients who are suffering from obesity: bathing with cold water, *Rasāyana*, fresh *Śālī* type of rice, wheat, such activities as lying or resting on luxury cushions, products of milk, products of sugarcane-jaggery, etc., fried nuggets prepared out of the *Urada* beans, taking huge amount of meals, *Snehana Karma* (massage), eating fish and meat, sleeping during daytime, sporting garlands and perfumed oil and sandalwood, eating sweet products, drinking lot of water after taking meals and *Vamana* therapy. (B.R. 39/72-74).

काश्य (Kārśya)

निरुक्ति एवं परिभाषा (Nirūkti and Paribhāṣa)

The term '*Kārśya*' is derived from the word '*Kṛśa*' meaning '*Tanūkaraṇe*' which means making thin or emaciated. Hence, the word '*Kārśya*' means emaciation of the body.

कृशस्य भावः काश्यः । *Śabdakalpdruma*

A factor which expresses the thinness or leanness or emaciation of the body is known as *Kārśya*.

काश्यं कृशकरणम् । *Śabdakalpdruma*

Skininess or slenderness or emaciation of the body is called *Kārśya*.

अपचयः काश्यः । *S.S.Su. 10/5 (Dalhaṇa)*

Diminution or decrease or degradation of the body is called *Kārśya*.

काश्यं मांसक्षयः ॥ *S.S.Su. 15/3 (Dalhaṇa)*

Diminution of muscle tissue is called *Kārśya*.

काश्यमिति कृशता शरीरस्य वातप्रकृतिजत्वात् ॥

Ādd.Śa.Prā. 15/3

Emaciation of the body due to predominance of *Vāta* is called *Kārśya*.

शुष्कस्निग्धगुदरग्रीवो

धमनीजालसन्ततः ।

त्वगस्थिशोषोऽतिकृशः स्थूलपर्वा नरो मतः ॥

C.S.Su. 21/15

Atikṛśa or *Kārśya* is characterised by emaciation of buttocks, abdomen and neck, prominence of network of vessels all over the body associated with leanness and bony prominence with large joints.

सम्प्राप्ति घटक (Samprāpti Ghaṭaka)

<i>Doṣa</i>	: <i>Vāta pradhāna tridoṣa</i>
<i>Dūṣya</i>	: <i>Rasa</i>
<i>Agni</i>	: <i>Jaṭharāgni māndhya</i>
<i>Āma</i>	: <i>Jaṭharāgni māndhyajanya</i> <i>Āma</i>
<i>Srotas</i>	: <i>Rasa, Māmsa and</i> <i>Medovaha</i>
<i>Srotoduṣṭi</i>	: <i>Saṅga</i>

Saṅcāra sthāna: Sarva Rasāyanis
Adhiṣṭhāna : Sarva Sarīra
Udbhava sthāna: Āmapakvāśaya
Vyakta sthāna : Sarvāṅga especially Sphik,
 Udara and Grīva
Rogamārga : Bāhya

कार्श्य निदान (Kārśya Nidāna)

तत्र पुनर्वातलाहारसेविनोऽतिव्यायामव्यवायध्वयनभयशोक-
 ध्यानरात्रिजागरणपिपाशाक्षुत्कषायाल्पाशनप्रभृतिभिरुपशोषितो
 रसधातुः शरीरमननुक्रामन्नल्पत्वात् प्रीणाति, तस्मादतिकार्ष्यं भवति,
 सोऽतिकृशः क्षुत्पिपासाशीतोष्णवातवर्षभारादानेष्वसहिष्णु-
 र्वातरोगप्रायोऽल्पप्राणश्च क्रियासु भवति, श्वासकासशोष-
 प्लीहोदराग्निसादगुल्मरक्तपित्तानामन्यतममासाद्य मरणमुपयाति ।
 सर्व एव चास्य रोगा बलवन्तो भवन्त्यल्पप्राणत्वात् ।
 अतस्तस्योत्पत्तिहेतुं परिहरेत् ॥ S.S.Su. 15/33

Due to indulgence of food which increase Vāta along with excessive physical exercises, sexual intercourse, study, fear, grief, worry, keeping awake at nights, suppression of thirst & hunger, consumption of astringent taste food, less intake of food etc. leading to less formation & dryness of Rasadhātu. The same Rasadhātu circulate all over the body fails to nourish the other Dhātus because of its inadequate quantity. This is known as Kārśya.

Kārśya people becomes incapable of tolerating hunger, thirst, cold, heat, breeze, rain, carrying heavy loads, Vātajā Vikara & less physical activities.

Diseases manifest quickly in Kārśya people are as follows : Śvāsa, Kāsa, Śoṣa, Plīha, Udara, Agnisāda, Gulma, Raktapitta, Arśa, Grahaṇi & lastly death. All the above diseases are serious because of less strength of the body.

कार्श्य लक्षण (Kārśya Lakṣaṇa)

शुष्कस्फिगुदरग्रीवाधमनीजालसन्ततिः ।
 त्वगस्थिशोषोऽतिकृशः स्थूलपर्वाननो मतः ॥

BPM 40/3

Emaciation, wasting of the buttocks, abdomen & neck, appearance of network of veins & tendons all over the body, prominence of skin, bones & joints.

Please refer first volume of the author for more information about Kārśya. Don't diagnose Sthoulya & Kārśya by seeing one's own body.

कश्चिदन्यः कृशोऽस्तीव बलवान्दृश्यते तदा ॥
 आधानसमये यस्य शुक्रभागोऽधिको भवेत् ।
 मेदोभागस्तु हीनः स्यात्स कृशोऽपि महाबलः ॥
 मेदसस्त्वधिको यस्य शुक्रभागोऽल्पको भवेत् ।
 स स्निग्धोऽपि सुपुष्टोऽपि बलहीनो विलोक्यते ॥

BPM 40/6

An emaciated person who at the time of his birth had more portion of Śukrabhāga & less amount of Medobhāga will be very strong though appears as emaciated.

On the contrary one who at the time of his birth had more portion of Medobhāga & less portion of Śukrabhāga will be poor in strength though those persons appear as were nourished & unctous.

साध्यासाध्यता (Sādhyāsādhyata)

कार्श्यमेव वरं स्थौल्यात् न हि स्थूलस्य भेषजम् ।
 बृंहणं लङ्घनं वाऽलमतिमेदोऽग्निवातजित् ॥

A.H.Sū. 14/31

स्थौल्यात् कार्श्यस्य सुखसाध्यत्वमाह ।

A.H.Sū. 14/31(Indu)

Reasonably Kārśya is easily curable than Sthoulya, because there is no treatment for Sthoulya; neither Br̥mhaṇa nor Laṅghana are enough to control Atimeda (Excessive fat), Agni (Digestive fire) and Vāta.

स्वभावादतिकार्ष्यो यः स्वभावादल्पपावकः । स्वभावादबलो यश्च-तस्य नास्ति चिकित्सितम् ॥ (B.P.M. 40/11)

There is no treatment to Kārśya if it occurs due to Svābhāva (Nature or birth), Svabhāvāda-lpapāvakaḥ (Whose digestive fire is weak) and Svabhāvādabalo (Whose strength is low).

Under-nutrition

Starvation and famine

1. There remain regions of the world, particularly rural Africa, where under-nutrition due to famine is endemic, the prevalence of BMI < 18.5kg/mg in adults is as high as 20%, and

growth retardation due to under nutrition affects 50% of children.

2. WHO reports that under-nutrition is responsible for more than half of all childhood deaths world-wide.
3. Starvation is manifest as marasmas (malnutrition with marked muscle-wasting); or kwashiorkor (malnutrition with oedema).
4. Growth retardation is due to deficiency of key nutrients, e.g. protein, zinc, potassium, phosphorus and sulphur.

Classification of under-nutrition in adults by body mass index (weight/height²)

BMI (kg/m ²)	Classification
> 20	Adequate nutrition
18.5-20	Marginal
< 18.5	Under-nutrition
17-18.4	Mild
16-17	Moderate
< 16	Severe

Causes of under-nutrition and weight loss in adults

Decreased energy intake

- Famine
- Persistent regurgitation or vomiting
- Anorexia, including anorexia nervosa
- Malabsorption (e.g. small intestinal disease)
- Maldigestion (e.g. pancreatic exocrine insufficiency) I

Increased energy expenditure

- Increased BMR (thyrotoxicosis, trauma, fever, cancer cachexia)
- Excessive physical activity (e.g. marathon runners)
- Energy loss (e.g. glycosuria in diabetes)
- Impaired energy storage (e.g. Addison's disease, pheochromocytoma)

Clinical assessment

1. In starvation, the severity of malnutrition can be assessed by anthropometric measurements as mentioned above.
2. Demispan and mid-arm circumference

measurements are most useful in monitoring progress during treatment.

The clinical features of severe under-nutrition in adults include:

- Weight loss
- Thirst, craving for food, weakness feeling cold
- Nocturia, amenorrhoea impotence
- Lax, pale, dry skin with loss of turgor and, occasionally, pigmented patches
- Cold and cyanosed extremities,

Pressure sores

- Hair thinning or loss (except in adolescents)
- Muscle-wasting, best demonstrated by the loss of the temporalis and periscapular muscle and reduced mid-arm circumference
- Loss of subcutaneous fat, reflected in reduced skin fold thickness and mid-arm circumference
- Hypothermia, bradycardia, hypotension small heart
- Oedema, which may be present without hypoalbuminaemia ('famine oedema')
- Distended abdomen with diarrhoea
- Diminished tendon jerks
- Apathy, loss of initiative, depression, introversion, aggression if food is near by
- Susceptibility to infections

Consequences of Under nutrition

1. Under-nutrition often leads to vitamin deficiencies, especially of thiamin, folate and vitamin C
2. Diarrhoea can lead to depletion of sodium, potassium and magnesium.
3. The high mortality rate in famine situations is often due to outbreak of infection, e.g. typhus or cholera, but the usual signs of infection may not be apparent.
4. In advanced starvation, patients become completely inactive and may assume a flexed, fetal position.
5. In the last stage of starvation, death comes quietly and often quite suddenly.
6. The very old are most vulnerable.

7. All organs are atrophied at necropsy, except the brain, which tends to maintain its weight.

Investigations

1. Plasma free fatty acids are increased and there is ketosis and a mild metabolic acidosis.
2. Plasma glucose is low but albumin concentration is often maintained because the liver still functions normally.
3. Insulin secretion is diminished, glucagon and cortisol tend to increase.
4. Reverse T_3 , replaces normal triiodothyronine.

5. The resting metabolic rate falls, partly because of reduced lean body mass and partly because of hypothalamic compensation.
6. The urine has a fixed specific gravity and creatinine excretion becomes low.
7. There may be mild anaemia, leucopenia and thrombocytopenia.
8. The erythrocyte sedimentation rate is normal unless there is infection.
9. The electrocardiogram shows sinus bradycardia and low voltage.

Differential diagnosis of Kārśya

S.No.	Kārśya	Dhātukṣaya
Type of Vyādhi	Apatarpanajanya Vyādhi & Kṛṣa pramehi	Apatarpanajanya Vyādhi
Etiology	Excessive consumption of rough foods and drinks • Fasting • Less intake of food • Excessive exposure to purificatory therapies • Grief • Suppression of natural urges including the urge for sleep • Disturbed sleep • Application of non unctuous substances in excess • Excessive indulgence in bath • By birth due to genes • Old age • Suffering from chronic disorder	The following diets are responsible for the depletion of dhatus c S Cī 1-2/3 Grāmya Āhara (domestic food and lifestyle) Grāmya Āhara (poor quality diet) is described below: • Excess intake of Amla (Sour), Lavaṇa (salt), Kaṭu (pungent) taste food • Excess intake of Kṣāra (alkaline), Suṣka Sāka (dry green vegetables), Suṣka Māmsa (dry meat), Tila/ Acurnata Tila (sesame seeds), Tila Palala/ Tila Curṇa (paste of sesame seeds and pastries) • Pistanna bhojana- food prepared from flour like cake and pastries • Viruddha Nava Suka Sami Dhānya (germinated cereals and pulses, freshly harvested corns with bristles and pulses) • Viruddha (intake of antagonistic food) • Asāthmya (unwholesome foods) • Klinna, Guru, Puti Paryusita Bhojana (softened, moist, heavy, putrid and stale food), • Viśamāśana (eating at irregular intervals) • Adhyāśana (eating too soon after a meal without the digestion of previous meal) Habits that cause illness - • Divāsvapna (day sleep) • Stree Madya Nitya (habitual indulgence in sex and alcohol) • Visama and Ati Vyāyama (irregular and excessive exercise),

		• <i>Bhaya</i> (fear), <i>Krodha</i> (anger), <i>Śoka</i> (grief), <i>Lobha</i> (greed), <i>Moha</i> (confusion), <i>Ayasa Bahula</i> (exhaustion due to overstrain).
Does not tolerate	Atikṛśa person does not tolerate व्यायाममत्तिसौहित्यं क्षुत्पिपासामयीषधम् । कृशो न सहते तद्वदतिशीतोष्णमैथुनम् । C.Su 21/13 • Physical exercise • Over saturation or excess consumption of food • Hunger, thirst • Diseases • Moderate and powerful drugs • Excessive cold and heat • Sexual intercourse Fail to perform normal physiological daily routines	
Diseases/effects	The following diseases manifest in atikṛśa individual प्लीहा कासः क्षयः श्वासो गुल्मोऽर्शास्युदराणि च । कृशं प्रायोऽभिधावन्ति रोगाश्च ग्रहणीगताः । C.Su 21/14 • <i>Pliḥā roga</i> • <i>Kāsa</i> • <i>Kṣaya</i> • <i>Śvāsa</i> • <i>Gulma</i> • <i>Arśa</i> • <i>Udara roga</i> • <i>Grahaṇi doṣa</i>	Effect of Grāmya Āhara Sevana - The intake of such <i>Grāmyāhara</i> (domestic food and Lifestyle) causes 1. <i>Sithilata</i>- slackening of muscle tissue compactness 2. <i>Vimucyante Sandhi</i> - Looseness/detachment of joint 3. <i>Vidahyate Rakta</i> - Blood gets inflamed & burnt 4. <i>Viṣyandate Alpa Meda</i> - increased free fatty acids 5. <i>Na Sandhiyate Asthiṣu Majja</i>- Marrow does not mature in bones 6. <i>Śukra Na Pravartate</i>-less formation of Semen 7. <i>Kṣaya Ojas</i>- Oja deteriorates
Clinical presentations	Prominent features of atikṛśa individual are : शुष्कस्निग्धगुदरग्रीवो धमनीजालसन्ततः । त्वगस्थिशेषोऽतिकृशः स्थूलपर्वा नरो मतः । C.Su 21/15 • Wasting prominently marked in buttocks, abdomen and neck • Prominent vascular net work in the body • Remnants of skin and bone • Joints are prominently seen	The person suffers from malaise, despair, hypersomnia and excessive sleepiness, drowsiness, lassitude, lack of enthusiasm, difficulty in breathing, unable to execute physical and mental activities, loss of memory, intellect and luster, becomes resort of illness and thus does not enjoy the normal life span
Pathogenesis	Develop due to Vatakopanidana which result in less formation and dryness of <i>Rasa dhātu</i> followed by deprivation of subsequent <i>dhātus</i>	Anuloma or pratiloma type of depletion of <i>Dhātus</i>

Weight Loss

Ask the Following Questions:

Is there fever? The presence of fever would suggest an infectious disease, such as tuberculosis, AIDS, brucellosis, and typhoid fever, but collagen diseases and neoplasms should not be forgotten.

Is there anorexia? The presence of anorexia may be related to a febrile process, but if there is no fever one should consider the possibility of Addison's disease, anorexia nervosa, Simmonds' disease, drug abuse, poisoning, such as arsenic poisoning, scurvy, malabsorption syndrome, uremia, and liver failure. There may also be a neoplasm.

Is there lymphadenopathy? The presence of generalized lymphadenopathy should suggest leukemia, sarcoidosis, and lymphoma, as well as infectious disease processes.

Is there an abdominal mass? An abdominal mass may be an enlarged spleen, a pancreatic carcinoma, an enlarged liver, or renal mass. These masses would suggest disease of those organs. The mass also may be a carcinoma of the stomach or intestine.

Is there hyperpigmentation? The presence of hyperpigmentation would suggest Addison's disease.

Is the appetite normal or increased? The presence of a normal or increased appetite in the presence of weight loss should suggest hyperthyroidism and diabetes mellitus. The patient also may be taking thyroid hormone medication in increased quantities.

Is the thyroid gland enlarged? The presence of an enlarged thyroid would suggest hyperthyroidism. One should also look for a focal thyroid mass which might be a toxic adenoma.

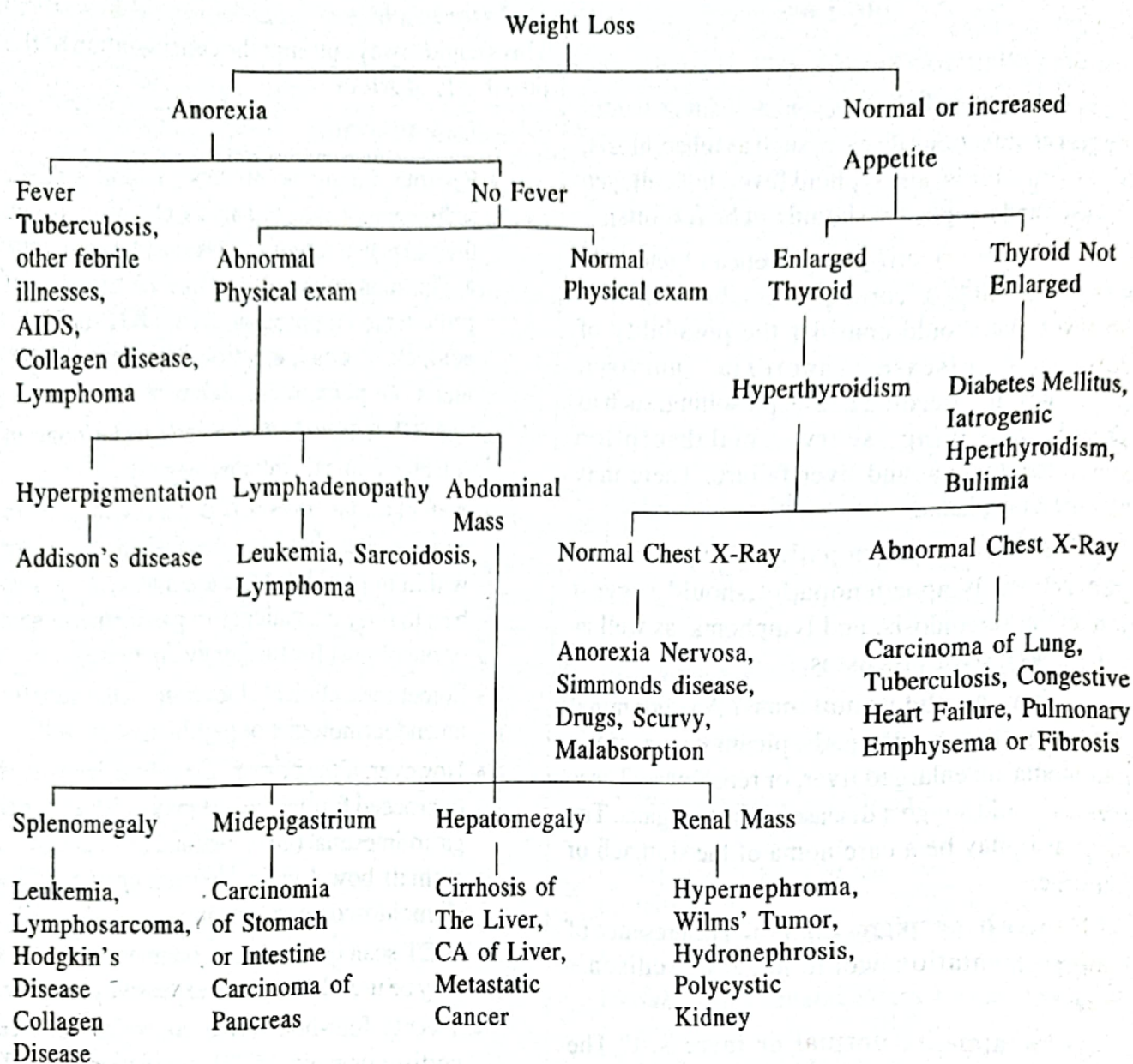
Is the chest x-ray abnormal? Abnormalities found on x-ray that may induce weight loss are carcinoma of the lung, tuberculosis, congestive heart failure, pulmonary emphysema, and fibrosis.

Is there a history of risky sexual behavior?

This should always prompt the consideration of the possibility of AIDS.

Diagnostic Workup

- Routine diagnostic studies include a CBC, sedimentation rate, urinalysis, chemistry panel, thyroid panel, serum amylase and lipase, febrile agglutinins, tuberculin test, ANA titer, serum protein electrophoresis, serum B12 and folic acid, chest x-ray, electrocardiogram (EKG), and a flat plate of the abdomen.
- An HIV antibody titer needs to be done in selected clinical circumstances.
- A stool for fat, trypsin, occult blood, and ovum and parasites should be done. If these tests are within normal limits or are unrevealing, it is best to refer the patient to a gastroenterologist or oncologist for further evaluation.
- Sometimes, clinical clues suggest the need for an endocrinologist or psychiatrist as well.
- However, if the primary care physician wishes to proceed further, he/she may order an upper gastrointestinal (GI) series and esophagogram, a small bowel series, barium enema, and a sigmoidoscopic examination.
- A CT scan of the chest, abdomen, and pelvis may be useful, but it is an expensive procedure.
- Twenty-four-hour urine collection for free cortisol or rapid ACTH stimulation test will diagnose Addison's disease and hypopituitarism.
- Quantitative stool fat and D-xylose absorption or a simple glucose tolerance test will diagnose some cases of malabsorption syndrome.
- Endoscopic procedures, including laparoscopy, and even an exploratory laparotomy, have their place in the diagnostic workup. However, it is always best to enlist the help of specialists before considering these procedures, even if one is located in an isolated community. If a trial of a nutritional supplement halts the weight loss, depression is most likely the cause.



	Fever	Appetite	Abdominal Mass	Polyuria	Lymphadenopathy
Infectious disease	Yes	Decreased	Infrequent	No	Yes
Neoplasm	No	Decreased	Often	No	Maybe
Lukemia	Occasional	Decreased	Maybe	No	Yes
Hyperthyroidism	No	Increased	No	Yes	No
Diabetes mellitus	No	Increased	No	Yes	No
Anorexia nervosa	No	Decreased	No	No	No
Bulimia	No	Normal	No	No	No

MCQs

1. आहारपरिणामान्तेभूयश्चलभतेबलम is clinical feature of
a. व्यपेताहिकका (Vyapeta Hikk)
1. "Medasavritamargatvadvyuhkoshtavishe-
shtah" explained in context to
a. Atisthoola b. Medasavritavata
c. Atisara d. Udararoga
2. Dhatu responsible for Karsya
a. Medas b. Mamsa
c. Rasa d. Sukra

3. तस्यह्यतिमात्रमेदस्विनोमेदएवोपचीयतेनतयेतरेधातवः _____
a. Krichravayavayata b. Ayushohrasa
c. Dourbalya d. Javoparodha
4. शैथिल्यात्सौकुमार्याद्दुरुत्वाच्चमेदसो _____
a. Dourbalya b. Javoparodha
c. Krichravayavayata d. Ayushohrasa
5. Asamatwa of Dhatus in Sthula result in
a. Dourbalya b. Javoparodha
c. Dourbalya d. Krichravayavayata

Answer

1.	a	2.	c	3.	b
4.	b	5.	c		